Nikki.

FREE GUIDE · CLINIC OWNERS & MANAGERS

The Clinic *Front-Desk* Fix

A 25-point checklist to cut no-shows and claim denials — built from real front-office work in academic medicine, dialysis and clinic operations.

01



Nikhila (Nikki) Mamanduri
Healthcare Operations Manager · MHA, CAPM

WHY I MADE THIS

Most revenue problems start at the front desk.

At Tufts Medicine I ran scheduling, insurance verification, prior authorizations and billing workflows. Small fixes at the front of the patient journey added up: throughput rose 25%, claim denials fell 20%, administrative bottlenecks dropped 28% and scheduling conflicts fell 15%.

None of that came from one big project. It came from making the same handful of steps happen the same way, every time — and making it obvious when they didn't. This checklist is that list.

How to use it. Walk through the 25 points with your front-desk lead. Tick what's already reliable, circle what isn't, and pick **three** circled items to fix in the next 30 days (there's a plan on page 4). Re-score monthly.

> *"Patient experience is not separate from operational leadership. It is one of the clearest signals of it."*

What "good" looks like

- Every appointment is **verified, authorized and confirmed** before the patient arrives.
- Patients know **what to bring, what it costs and what happens next**.
- Problems surface as **exceptions on a list**, not surprises at check-in or on a denial letter.
- The team works from **written SOPs**, so quality doesn't depend on who's at the desk today.

01 Scheduling & reminders

- [ ] **One scheduling template** per provider, with visit types, lengths and buffers written down.
- [ ] New-patient slots are **protected** and released to follow-ups only 48 hours out.
- [ ] Every booking captures **reason for visit, insurance and preferred contact method**.
- [ ] **Two reminders** go out (e.g., 3 days and 1 day before) by text and/or call — with a one-tap confirm or reschedule.
- [ ] A **same-day waitlist** is used to backfill cancellations within the hour.

02 Insurance verification & prior authorization

- [ ] Eligibility is **verified for every visit**, not just new patients — ideally 2–3 business days ahead.
- [ ] A **prior-auth list by payer and service** is kept current and posted where schedulers can see it.
- [ ] Auth requests have an **owner, a due date and a status** on a shared tracker.
- [ ] Visits with **missing or pending auth are flagged** 48 hours out and the patient is called.
- [ ] Coverage changes (new plan, new year, secondary insurance) **trigger re-verification**.

03 Check-in & intake

- [ ] Forms and consents are **sent digitally before the visit**; paper is the backup.
- [ ] Photo ID and insurance card are **scanned and matched** to the chart every time.
- [ ] Demographics are **confirmed out loud**: name spelling, DOB, address, phone, guarantor.
- [ ] Copays and balances are **collected at check-in** with a friendly, scripted ask.
- [ ] Check-in time is **tracked**; anything over target is reviewed weekly.

04 Charge capture & claims

- [ ] Every visit is **closed and coded the same day**; open encounters are reviewed each morning.
- [ ] **CPT/ICD-10 codes match the documentation**; common mismatches are on a cheat sheet.
- [ ] Claims are **scrubbed before submission** for missing modifiers, auth numbers and demographics.
- [ ] Denials are **logged by reason code** and worked within a set number of days.
- [ ] The **top three denial reasons** are reviewed monthly and fixed at the source.

05 Follow-up & recall

- [ ] Patients **leave with their next appointment booked** when follow-up is needed.
- [ ] No-shows get a **same-day call** and a reschedule offer, not just a note in the chart.
- [ ] A **recall list** (overdue follow-ups, lapsed treatment plans) is worked weekly.
- [ ] Patient feedback and reviews are **read and shared with the team** every week.
- [ ] Every recurring problem gets an **SOP update** so it doesn't depend on memory.

MEASURE IT

Six numbers to watch every week

You can't fix what you can't see. Put these on one simple dashboard (a spreadsheet is fine) and review them in a 10-minute weekly huddle.

KPI	How to calculate	Why it matters
No-show rate	No-shows ÷ scheduled visits	Lost capacity and delayed care.
Same-day cancellation rate	Cancellations <24h ÷ scheduled visits	Shows where reminders and waitlists help.
First-pass claim acceptance	Claims paid on first submission ÷ claims submitted	The best single signal of front-end quality.
Denial rate (by reason)	Denied claims ÷ claims submitted	Points to the step that needs fixing.
Auth turnaround	Days from request to approval	Late auths cause reschedules and denials.
Check-in time	Arrival to roomed (minutes)	What patients feel first.

Your 30-day plan

Week	Focus
1	Score the checklist with your team. Pick 3 circled items. Start tracking the six KPIs.
2	Write or update the SOP for each of the 3 items. Assign one owner per item.
3	Run it. Hold a 10-minute daily huddle on exceptions (missing auths, unconfirmed visits).
4	Compare KPIs to week 1. Keep what worked, adjust what didn't, choose the next 3.

Tip: start with the items that touch every visit — eligibility checks, reminders and same-day coding. They usually move the numbers fastest.

NEXT STEP

Want a second pair of eyes on *your* front desk?

I'm happy to walk through your checklist results with you. Book a free 15-minute intro call, or a working session if you'd like a deeper review of your scheduling, authorization or billing workflow.

Session	Length	Price	Best for
Intro call	15 min	Free	Say hello and see if we're a fit.
Mentoring chat	30 min	Free	Career questions, MHA advice or a resume review.
Working session	60 min	On request	A focused session on your clinic or project.

Save my digital card

Scan to save my contact, book a call from my website, or connect on LinkedIn: linkedin.com/in/nikki-mamanduri

About Nikki. Nikhila (Nikki) Mamanduri is a Healthcare Operations Manager at Medi-Weightloss® Thousand Oaks with an MHA from the University of New Haven (3.79 GPA) and PMI's CAPM®. She has worked across medical devices (Relisys), dialysis (U.S. Renal Care), academic medicine (Tufts Medicine) and clinic operations, and believes better systems create stronger teams — and a better experience for every patient.

This guide shares general operational practices for educational purposes. It isn't legal, billing-compliance, immigration or medical advice — check payer rules, your organization's policies and the relevant authorities for your situation.